

Copyable SMART goals for hand therapy documentation.

By Joseph Sharaya, OTD, OTR/L, CHT · Certified Hand Therapist · Updated September 2026

Before you copy a goal: the numbers are starting points, not norms. All numerical targets on this page — Targets Hit, Time to Target, and Path Efficiency thresholds — are clinically reasonable starting points, not published normative cutoffs. WristSkill™ does not yet have published normative data. Adjust targets to the patient in front of you.

Each goal below is written in the standard format used in clinical documentation: the patient, the measurable performance indicator, the WristSkill metric involved, the timeframe, and the functional occupation-based outcome. Goals are framed with the Occupational Therapy Practice Framework (OTPF-4) in mind: every goal connects WristSkill performance data to a specific ADL, IADL, work task, or play/leisure activity the patient needs to resume.

Clinicians using WristSkill should adapt these goals to the specific patient, stage of rehabilitation, surgical stabilization status, and physician clearance. The goals reference the three WristSkill metrics — Targets Hit, Time to Target, and Path Efficiency — across the six movement planes. Organization is by injury type.

On the movement planes. WristSkill's six planes are not isolated cardinal motions. Because the phone is held in the hand, forearm pronation and supination contribute to every plane alongside wrist flexion-extension and radial-ulnar deviation. The plane labels describe the dominant wrist motion; forearm rotation is always part of the picture.

Clinical reminder. The motion recommendations for each injury type reflect current evidence but are not a substitute for clinical judgment. Individualize to the patient in front of you.

Goals for SL injury

Favored WristSkill motions: radial deviation (Horizontal), D2 extension phase, wrist flexion (Vertical).

Use with caution: wrist extension, ulnar deviation, D2 flexion. Circumduction is reserved for advanced stages. Remember that DTM is not unconditionally safe with an unstable SL; limit to mid-range arcs.

STAGE 2 · PROPRIOCEPTION AWARENESS (WEEKS 4–8)

- Patient will reach at least 6 targets per trial in the Horizontal plane with Path Efficiency $\geq 70\%$, within 4 weeks, to support return to light self-care ADLs including facial grooming, eating with utensils, and donning a shirt without compensatory shoulder substitution.
- Patient will demonstrate symmetry in the Horizontal plane between involved and uninvolved wrist (within 3 targets), within 4 weeks, to support return to bilateral IADL tasks such as folding laundry and meal preparation with two-handed carrying.

STAGE 3 · CONSCIOUS NEUROMUSCULAR CONTROL (WEEKS 8–12)

- Patient will achieve Path Efficiency $\geq 75\%$ in the D2 plane on the involved side, within mid-range arc only, within 6 weeks, to support return to fine-motor IADLs such as fastening buttons, handling coins, and typing on a keyboard for sustained sessions.
- Patient will demonstrate Time to Target < 3.0 seconds in the Vertical plane within 4 weeks, to support return to pushing and pulling tasks such as opening refrigerator doors, pushing a shopping cart, and transitional bed mobility.

STAGE 4 · ADVANCED (WEEK 12+)

- Patient will complete CircleCW and CircleCCW trials with Path Efficiency $\geq 70\%$ and minimum 5 targets per trial, within 8 weeks, to support return to work demands requiring multi-plane wrist control such as carrying tools, operating hand controls, and sustained computer use.
- Patient will demonstrate full Stage 3 symmetry against the uninvolved side within 12 weeks, to support return to play and leisure activities such as gardening, yoga, and recreational sport.

Goals for LT injury

Favored WristSkill motions: ulnar deviation (Horizontal), wrist extension (Vertical), D1 extension phase (ulnar extension, the ECU direction) early, then both D1 phases, the reverse dart-throwing motion, later. Use with caution: excessive supination torques, and the D2 flexion phase (radial extension works ECRL and ECRB, carpal supinators). Strengthening focus: ECU in forearm neutral early, then combined ECU and FCR activation in the reverse dart-throwing motion; avoid training the carpal supinators (ECRL, ECRB, APL, FCU) (Hagert & Rein, 2024).

STAGE 2 · PROPRIOCEPTION AWARENESS (WEEKS 4–8)

- Patient will demonstrate Time to Target < 4.0 seconds in the Horizontal plane with forearm in neutral rotation, within 4 weeks, to support return to ADLs such as drinking from a cup, holding a phone to the ear, and using a door handle.
- Patient will reach at least 5 targets in the Vertical plane per trial with Path Efficiency $\geq 65\%$, within 4 weeks, to support return to grip-and-lift IADLs such as carrying a grocery bag, lifting a small pan, and opening cabinet doors.

STAGE 3 · CONSCIOUS NEUROMUSCULAR CONTROL (WEEKS 8–12)

- Patient will achieve Path Efficiency $\geq 70\%$ in the D1 plane within 6 weeks, to support return to work tool handling such as using a screwdriver and sustained power-grip use.
- Patient will achieve symmetrical Targets Hit in the D1 plane with < 3 targets difference from uninvolved side, within 8 weeks, to support return to diagonal reaching IADLs such as reaching into an overhead cabinet or washing the opposite shoulder.

STAGE 4 · ADVANCED (WEEK 12+)

- Patient will complete CircleCCW trials with Path Efficiency $\geq 70\%$ within 10 weeks, to support return to work or sport demands requiring integrated ulnar-dominant wrist rotation such as sustained keyboard use with mouse work, racquet sports, and manual trades.

Goals for TFCC injury

Favored WristSkill motions: ulnar deviation (Horizontal), wrist extension (Vertical), D2 extension. Use with caution: circumduction (assess DRUJ stability first), loaded pronation, combined wrist and end-range forearm rotation. Strengthening focus: pronator quadratus isometrics in forearm neutral; with the ECU, the pronator quadratus dynamically stabilizes the DRUJ (Hagert & Rein, 2024). In one study of deep-fiber TFCC tears, position sense deficits were found in forearm rotation, particularly pronation (Park et al., 2018).

STAGE 2 · PROPRIOCEPTION AWARENESS (WEEKS 4–8)

- Patient will complete the Horizontal plane assessment with forearm supported in neutral rotation, achieving Time to Target < 3.5 seconds within 4 weeks, to support return to ADLs in a supported-forearm position such as feeding with elbow on the table and reading with a book holder.
- Patient will tolerate the Vertical plane assessment without reproduction of ulnar-sided symptoms, within 4 weeks, to support progression to light IADLs such as pouring from a lightweight pitcher and folding lightweight clothing.

STAGE 3 · CONSCIOUS NEUROMUSCULAR CONTROL (WEEKS 8–12)

- Patient will achieve Path Efficiency $\geq 70\%$ in the D2 plane within 6 weeks, in forearm-neutral position, to support return to IADLs such as preparing a simple meal, drinking from a standard cup, and keyboard and mouse work in a neutral posture.
- Patient will demonstrate Targets Hit within 3 of uninvolved wrist in the Horizontal plane within 8 weeks, with DRUJ stability confirmed during clinical exam prior to advancing to circumduction, to support return to two-handed tasks such as opening a jar and wringing a towel.

STAGE 4 · ADVANCED (WEEK 12+)

- Patient will complete CircleCW and CircleCCW trials, tolerated without ulnar-sided pain and with Path Efficiency $\geq 65\%$, within 10 weeks, to support return to occupational tasks requiring multi-plane wrist rotation such as sustained driving, push-and-pull industrial tasks, and recreational activities involving rotational loading.

Goals for DRF

Favored WristSkill motions: all six planes are appropriate; D2 (the DTM plane) is particularly functional. After volar plate fixation, early active motion gives better short-term function than delayed mobilization and similar results at one year (Gutiérrez-Espinoza et al., 2021), provided surgical stabilization is adequate. Early motion may also carry a higher rate of implant loosening or fracture re-displacement (Deng et al., 2021), so start with surgeon clearance. Key assessment emphasis: JPS and motor performance respond strongly to intervention in this population.

STAGE 2 · PROPRIOCEPTION AWARENESS (WEEKS 4–8)

- Patient will complete all six WristSkill planes on the involved side with at least 4 targets per plane, within 4 weeks, to support return to basic self-care ADLs including dressing, grooming, and toileting with the involved hand.
- Patient will demonstrate Path Efficiency $\geq 60\%$ in the Horizontal and Vertical planes within 4 weeks, to support return to light IADL re-engagement including handling a mug, turning a key, and opening lightweight packaging.

STAGE 3 · CONSCIOUS NEUROMUSCULAR CONTROL (WEEKS 8–12)

- Patient will achieve Path Efficiency $\geq 75\%$ in the D2 plane within 6 weeks, to support return to dart-thrower's motion functional tasks including hammering, pouring from a standard pitcher, casting a fishing rod, and occupational tool handling.
- Patient will reduce Time to Target in the involved wrist to within 0.5 seconds of the uninvolved wrist across the Horizontal, Vertical, D1, and D2 planes, within 8 weeks, to support return to typing and writing speed for work tasks.
- Patient will demonstrate at least 8 Targets Hit per plane across all four linear planes within 8 weeks, to support return to IADLs requiring pushing, pulling, and carrying loads up to 5 pounds.

STAGE 4 · ADVANCED (WEEK 12+)

- Patient will complete all six WristSkill planes symmetrically (within 2 targets and 0.3 seconds of uninvolved side), within 12 weeks, to support return to pre-injury occupation, including sustained computer work, manual trades, or caregiving roles involving lifting and transfers.
- Patient will demonstrate Path Efficiency consistent with pre-injury estimates in all six planes within 16 weeks, in conjunction with PRWE and grip strength symmetry, to support return to recreational activities and sport at pre-injury participation level.

Numbers remain starting points. As noted at the top of the page, all numerical targets in these goals are clinically reasonable starting points, not published normative cutoffs. Adjust based on patient presentation and clinical judgment.

References

- 1 Deng, Z., Wu, J., Tang, K., Shu, H., Wang, T., Li, F., & Nie, M. (2021). In adults, early mobilization may be beneficial for distal radius fractures treated with open reduction and internal fixation: A systematic review and meta-analysis. *Journal of Orthopaedic Surgery and Research*, 16(1), Article 691. <https://doi.org/10.1186/s13018-021-02837-0>
- 2 Gutiérrez-Espinoza, H., Araya-Quintanilla, F., Olguín-Huerta, C., Gutiérrez-Monclus, R., Jorquera-Aguilera, R., & Mathoulin, C. (2021). Effectiveness of early versus delayed motion in patients with distal radius fracture treated with volar locking plate: A systematic review and meta-analysis. *Hand Surgery and Rehabilitation*, 40(1), 6–16. <https://doi.org/10.1016/j.hansur.2020.10.007>
- 3 Hagert, E., & Rein, S. (2024). Wrist proprioception — An update on scientific insights and clinical implications in rehabilitation of the wrist. *Journal of Hand Therapy*, 37(2), 257–268. <https://doi.org/10.1016/j.jht.2023.09.010>
- 4 Park, J. H., Kim, D., Park, H., Jung, I., Youn, I., & Park, J. W. (2018). The effect of triangular fibrocartilage complex tear on wrist proprioception. *Journal of Hand Surgery*, 43(9), 866.e1–866.e8. <https://doi.org/10.1016/j.jhsa.2018.01.022>